

**Public Health Emergency Operations Center
"COUSP DRC"**

MVE-17 Incident Management System

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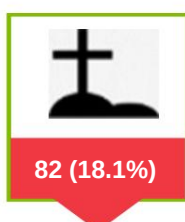
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°021/MVE BUNDIBUGYO_04/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (25)	• Ituri (17/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara • North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha • South Kivu (1/34): Miti-Murhesa
Reporting date	June 4, 2026
Publication date	June 5, 2026



**Total confirmed cases
for the 3 provinces**



**Cumulative deaths
among confirmed cases
and case fatality rate**



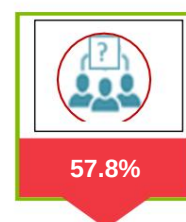
**Patients in
isolation -
hospitalization**



**Cumulative number
of recoveries**



**New suspected
cases today**



**Contact tracing
rates for the 3
provinces**

**Data currently being harmonized*

1. HIGHLIGHTS

- **Seventy-one (71) new confirmed cases, including 21 deaths**, were reported as of June 4, 2026, in the provinces of **Ituri** (65 confirmed cases, including 16 deaths) and **North Kivu** (6 confirmed cases, including 2 deaths). These cases are located in **12 health zones**, including 10 health zones in Ituri province and 2 health zones in North Kivu province.
- Effective start of sample analysis in the Mongbwalu health zone (equipment installed and personnel trained).
- Deployment of experts in the Mongbwalu health zone to assess continuity of care in health care facilities (HCFs).
- A confirmed patient of Ebola virus disease has been declared cured in the Aru health zone (Ituri province).

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

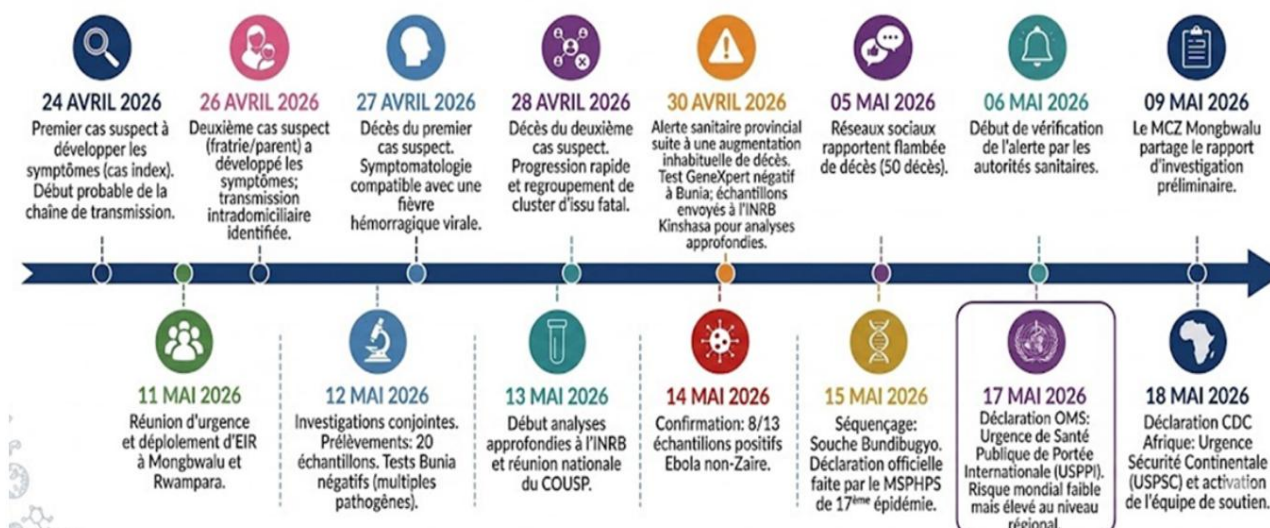


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 4, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	424	66	15.6%	17 out of 36 (47.2%)	65
North Kivu	25	15	60.0%	7 out of 34 (20.6%)	6
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	452	82	18.1%	25 out of 104 (24.0%)	71

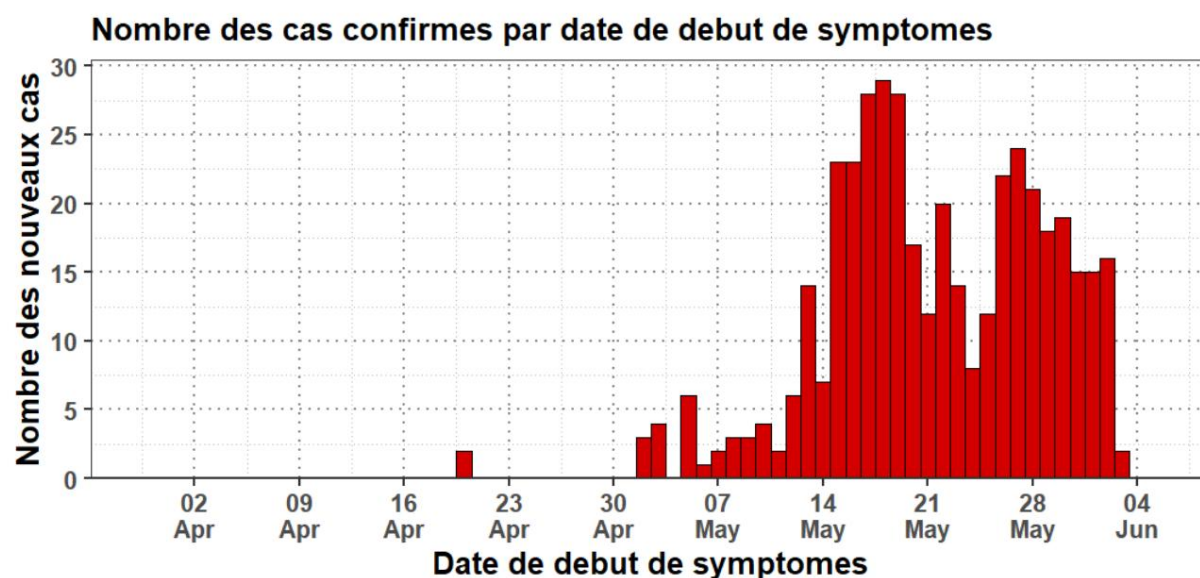
- Ituri remains the national epicenter with 94.0% of confirmed cases. The overall case fatality rate in the three provinces remains low, particularly in Ituri (15.6%), while it is high in North Kivu province (60.0%). The number of confirmed cases (n=71) recorded in the two provinces indicates rapid and continuous community transmission.
- North Kivu has a worrying case fatality rate of 60.0%, which can be explained by delays in care and escape of three confirmed cases from care facilities.

3.2 Distribution by health zone (Cumulative as of 04/06/2026)

- **Ituri Province (424 cases):** Bunia (123), Rwampara (86), Mongbwalu (64), Nyankunde (22), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damas (3), Kilo (3), Nizi (5), Mangala (1), Aungba (1), Gety (1), Komanda (3), Lita (3), Logo (2). Ninety-four (94) additional cases are being ventilated.
- **North Kivu Province (25 cases):** Katwa (11), Beni (5), Butembo (4), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time analysis (missing data from June 4, 2026)

A significant number of confirmed cases (cohort 1) were symptomatic between May 14 and May 23, 2026, suggesting increased contamination from a likely common outbreak, with a peak observed on May 18, 2026. Subsequently, a number of confirmed cases (cohort 2) developed symptoms between May 25 and June 3, demonstrating a spread of the disease and potentially constituting a significant reservoir; an increase in cases may be recorded if appropriate measures are not implemented very quickly.



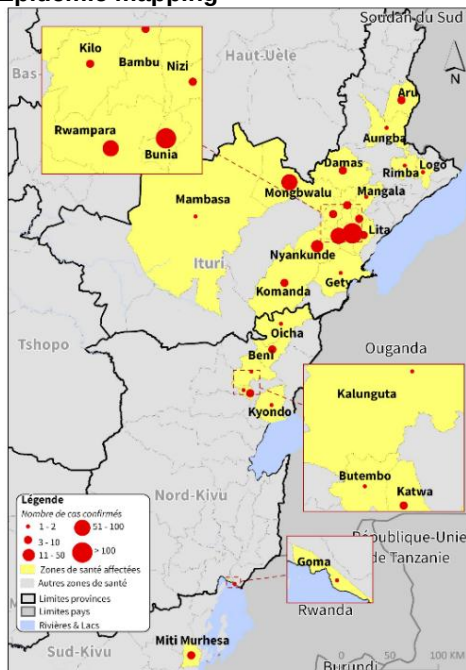
Source : DHIS2 Tracker - Riposte MVE, mai 2026. Données incluses dans l'analyse : n cas confirmés = 423.

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 4, 2026

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI			
Bunia	123	11	8.9%
Rwampara	86	18	20.9%
Mongbwalu	64	21	32.8%
Nyankunde	22		4.5%
Bamboo	5	1	40.0%
Aru	3		33.3%
Kilo	3		33.3%
Nizi	5	2	0.0%
Mangala	1	1100	0.0%
Damascus	3	0	0.0%
Aungba		0	0.0%
Gety		0	0.0%
Komanda	1	0	0.0%
Lita	1	0	0.0%
Logo	3	0	0.0%
Mambasa	3		50.0%
Rimba	2	1	0.0%
Other ZS (undisclosed data)	23	0	10.6%
Ituri subtotal	94 424	10 66	15.6%
NORTH KIVU			
Katwa	11	7	63.6%
Blessed	5	3	60.0%

Butembo	4	2	50.0%
Oicha	2	2	100.0%
Kalunguta			100.0%
Kyondo	11	10	0.0%
Goma			0.0%
North Kivu subtotal	125	015	60.0%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	452	82	18.1%

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 4, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

- **National / Ituri:**
 - Debriefing of the coordination team and the various technical and financial partners on the mission of the Nizi ZS, recently assigned.
 - Deployment of a joint mission in the Mongbwalu health zone to assess the establishments of care on the implementation of continuity of care.
- **North Kivu:** The Incident Management System (IMS) meeting was held at the Provincial Health Division (DPS) to review the evolution of the epidemic and operational priorities.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 4, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Alerts raised	146	236	12	394
Alerts investigated	133	215	11	359
Investigation rate	91.1%	91.1%	91.7%	91.1%
Alerts confirmed	104	44	5	153

Table 3. Contact tracing of confirmed cases as of June 4, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	3,997	2,181	54.6%
North Kivu	589	424	72.0%
South Kivu	180	150	83.3%
Total	4,766	2,755	57.8%

- Routine field surveillance activities (reporting alerts, investigations, active searching, listing and monitoring contacts, retrospective review of cases and deaths, etc.)
- In-depth investigations into confirmed cases in the affected health zones (Bunia, Nyakunde and Rwampara).
- Persistence of poor performance in Ituri (54.6%), roughly similar to the last 24 hours while in North Kivu (72.0%), there is an improvement in monitoring compared to the previous day.

4.3 Laboratory

- **Ituri:** 199 samples collected and analyzed (32.6% positivity rate); Deployment of a machine in the Mongbwalu health zone and effective start of sample analysis.
- **North Kivu :** 57 samples collected and analyzed, of which 6 tested positive for Ebola virus disease, but 118 Results are still pending.
- **South Kivu:** 2 samples received at the laboratory (1 re-sampling).

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Briefing of 16 TS (CM la Divinité) and 9 Care Providers (CME Nyakunde Bunia) on hand washing, wearing and removing PPE, bio-cleaning and waste management (Sanitation of BCZ); briefing of 5 providers in the PS Umoja (AS Lembabo) and 6 care providers on Bio-cleaning, wearing and removing PPE and waste management 4 EDS carried out on 4 alerts (HGR-Bunia).
- **North Kivu:** Alarming average PCI score of 22.9%; decontamination of the isolation room of the patient discharged after recovery at Heal Africa Hospital (Goma Health Zone), 2 households and 5 ESS in the Katwa and Butembo Health Zones; completion of 6 EDS including 3 in Katwa and 3 in Beni; briefing of 56 healthcare providers on standard precautions in the Katwa Health Zone.
- **South Kivu :** Scorecard evaluation of 2/30 ESS in the Miti Muhesa health zone, including 44% for Kachucha Hospital and 56% for Kavumu Hospital; provision of IPC inputs (100 handwashing kits, including 40 at ESS and 60 in the community, 40 decontamination kits in the Mitimurhesa and Katana health zones); briefing of 25 providers (IT and Doctors) on community and clinical IPC; decontamination of 2 ESS (Sodeseka private medical center in the Katana health zone) around the suspected high-risk case who died at Katana General Referral Hospital.

4.5 Holistic care

Table 4. Patient movement within healthcare facilities as of June 4, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	219	8	6	233
Total admissions (24h)	61	1	0	62
Exits (24h)				
Deceased (Suspects/Confessions)	6	0	0	6
No case / Cured	17	0	0	17
Escapees (Suspects/Confessions)	14	0	0	14
Total outflows	37	0	0	37
Patients in isolation (End of Day),	243	9	6	258
including confirmed cases	66	5	1	72
(NC+AC) and suspected cases	177	4	5	186

Mental and psychosocial health activities

- Psychological preparation of an Ebola survivor identified as a caregiver to facilitate medical care and psychosocial support for six (6) children aged 5 months to 2 years, all suspected cases from the Saint-Nicolas orphanage in the Bunia Health Zone.

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
Proportion of newly confirmed cases that benefited from SMSPS interventions	3 (27.3%)	0	0
Proportion of confirmed cases under follow-up that benefited from SMSPS interventions	18 (48.6%)	2 (100%)	3 (100%)
Proportion of new suspected cases that benefited from SMSPS interventions	24 (100.0%)	22 (78.6%)	3 (100%)
Proportion of suspected cases under follow-up that benefited from SMSPS interventions	49 (32.7%)	63	38 (100%)
Proportion of laboratory results having been announced	157 (47.6%)	1	19 (50%)
Including positive ones	70 (44.6%)		
Proportion of children separated in daycare who benefited from SMSPS interventions	0 (0.0%)	0	0
Proportion of children separated in the community who benefited from SMSPS interventions	6 (100.0%)	10	0
Number of accompanying persons at the CTE who benefited from SMSPS interventions	18.0	14	0
Number of PPLs who benefited from SMSPS interventions	27.0	14	5
Number of households and communities that benefited from SMSPS interventions	38.0	38	10
Proportion of patients who received Dignity Kits at the CTE	0.0	0	0

4.6 CREC and Community Engagement

- **North Kivu:** 16,132 people sensitized during door-to-door visits in the 3 health zones; 9,260 households visited thanks to the involvement of 917 RECOs out of 1,923 planned; 2,268 students and 93 teachers sensitized on Ebola prevention; 130 religious leaders briefed on prevention and community engagement; 46 community alerts reported and 1 rumor clarified; 18 local radio stations engaged with 3 formats produced and broadcast.
- **South Kivu:** Advocacy with 17 religious leaders in the Katana health zone on their involvement in the fight against Ebola; briefing of 80 community relays and members of the CACs of the Lwiro, Buhandahanda and Kavumu health areas on Ebola, the reporting of alerts and community feedback; mass awareness at the Kahaya primary school in the Kavumu AS in Miti-murhesa with 382 schoolchildren including 164 girls on prevention measures and signs of Ebola virus disease.
- **Ituri:** Speaking to the local press, the Director General of the INSP addressed the positive management of rumors, adherence to EDS practices, referral of individuals exhibiting symptoms of the disease to healthcare facilities, and the importance of an Ebola Treatment Center (ETC). **Fifty** internally displaced persons (IDPs) at the Kigonze site in the Bunia Health Zone were sensitized to the importance of EDS following a death. **As a result**, they expressed a need for a coffin. A community dialogue was also held in the Bunia Health Zone with 21 leaders from various religious denominations who had received educational messages on Ebola prevention within church and community settings.

4.7 Logistics and Security

- Delivery of PCI inputs to HGRs (Drodro, Lita, Mangala, Jiba, Damas, Kilo, Nizi, Bambu, Linga; Nyarambe, Rimba, Logo Angumu, Mahagi) and to the Institutes of Medical Techniques (Adventist, Itmdrodro, Nyakunde, Lolwa, Mungbwalu, Aradali, Bunia).
 - Continued delivery of PCI supplies to schools: EP mudzi pela, isp bunia, IDAP.
 - Delivery of PCI inputs •
- Provision of two additional vehicles to the PCI/EDS pillar supporting FHI 360.
- One vehicle was refueled.

4.8. Security

- The security situation remains unstable in the health zones of Nizi and Bambu.
- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the presence of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.9 PSEA (Prevention of Sexual Exploitation and Abuse)

- Start of PSEA activities as part of the response in the Miti Murhesa/South Health Zone Kivu
- Awareness-raising on the prevention of EAHS in the Miti Murhesa health zone, including 112 people (84 women and 27 men).
- Continued briefing of pillar members not briefed and signing of the code of conduct: 14 people including 11 men and 4 women.
- Updated database on the cumulative number of briefings and signing of commitment acts for good conduct is 951 people including 501 men and 410 women, 22 girls and 18 boys in Bunia, Rwampara and Monbgwalu.
- Briefing of CAC members and RECOs from the Miti Murhesa Health Zone in the DPS South Kivu on EAS concepts, reporting mechanisms, followed by the signing of the code of conduct. A total of 74 people participated, including 42 men and 32 women.
- Meeting held to plan activities for briefing and capacity building for focal points in the Ituri Health District Health Zones.

5. MAIN CHALLENGES

Challenge No.	Impact	Action required
Reluctance to perform post - mortem sampling (swabs)	Delay in confirmation; underestimation of cases	Intensive community engagement; involvement of religious leaders
2. Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Acceleration of the construction of planned CTES
3. Weakness in contact tracing (overall rate 45.5%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4. Supply failure in Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for the allocation of MEG
5. Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6. Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7. Insufficient resources financial for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Due date	
		Responsible pillar	
1	Training of service providers on the use of PCR RADIONE	Laboratory	Day 2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC	Day 3
3	Briefing of community health workers on Ebola virus disease surveillance and SBC (Ituri province)	Monitoring/CREC	Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri (current rate 41.2%)	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Catch-up plan for the 42 samples awaiting analysis in North Kivu	Laboratory	Day 2
8	North Kivu: discharge of the recovered case to Goma (Day 1), response to security incidents (ADF incursion), IPC training at the General Referral Hospital Virunga, CREC extension to 15,000 households		Day 2
9	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for IT/ECZS		Day 2

Press briefing by the Director General of INSP with the local press in Bunia, Ituri province.



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ADDITIONAL,
PLEASE CONTACT**

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